

Policy on Defined and Displayed Healthcare Services

«Hospital Name»

Document control

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Prepared by	Quality Coordinator Name: _____ Signature: _____		
Reviewed by	Quality Coordinator Name: _____ Signature: _____		
Approved by	Medical Superintendent Name: _____ Signature: _____		

A blank marked _____ must be completed before issue.

Statement of intent

Patients, families and referrers must know what this hospital provides — services defined with the community in mind, staffed and equipped, scoped by department, and displayed where people can read them.

1. Purpose

This policy says how «Hospital Name» defines the healthcare services it provides in consonance with community needs; ensures each defined clinical service has diagnostic and treatment capability with suitably qualified personnel for out-patient, in-patient, day-care and emergency cover; defines the scope of clinical services of each department; and prominently displays those defined clinical services.

The chapter intent is that patients are informed of the services provided, that the scope of each healthcare service including diagnostic and therapeutic services is well defined and available to patients and families, and that only patients the organisation can care for are admitted.

This policy does not cover patient registration, admission, transfer, referral, laboratory services, or imaging services — those are covered in other hospital policies.

2. Scope

This policy applies to every clinical and diagnostic department at «Hospital Name». It binds the Medical Superintendent, Heads of departments (broad speciality, super speciality and

diagnostic), the Quality Coordinator, registration and front-office staff, and clinical staff who deliver or describe services.

It covers defining healthcare services in line with community needs; diagnostic and treatment capability with qualified personnel including day-care and emergency cover; department scope, including outsourced clinical and diagnostic services; and permanent bi-lingual display.

Boundaries:

- Registration, admission and acceptance against the services defined here are covered in other hospital policies.
- Laboratory and imaging service detail is covered in other hospital policies. This policy owns that those services appear in the department scope and display.
- Patient-rights displays are covered in other hospital policies. This policy owns the clinical-services display.
- Outsourced services are named and known under this policy; the contract and quality oversight of those vendors sit with the owning clinical or diagnostic policy.

3. Policy standards

«Hospital Name» defines the healthcare services it provides, in consonance with the needs of the community it serves. Each defined clinical service has diagnostic and treatment services with suitably qualified medical, nursing and paramedical personnel who provide out-patient, in-patient, day-care and emergency cover. The scope of clinical services of each department is defined. The defined clinical services are permanently and prominently displayed in at least two languages.

«Hospital Name» does not claim a clinical service it cannot staff, diagnose for or treat, and does not leave the display out of date or unreadable.

4. Non-negotiable rules

1. Do not claim a clinical service the hospital cannot provide with qualified personnel and diagnostic and treatment capability for the care settings that service covers.
2. Do not operate a department whose clinical scope has not been defined and approved by the Medical Superintendent.
3. Do not omit documentation of clinical or diagnostic services that are outsourced; staff must know what is outsourced.
4. Do not remove the clinical-services display from a location visible to patients and visitors, allow it to become temporary when a permanent display is required, or leave it in only one language.
5. Staff who find the displayed services differ from what is actually provided report it the same shift to the Quality Coordinator or the Medical Superintendent.

5. What we do

5.1 Define healthcare services in consonance with community needs

The Medical Superintendent, with Heads of departments, defines the healthcare services «Hospital Name» provides. Senior management owns the definition. Community needs are considered when planning new services — captured through patient and family feedback, referral patterns, district disease-burden data, and changing disease patterns. Starting a new service on the hospital's own judgment is allowed; community need still informs the decision.

The defined services are recorded in the service directory of «Hospital Name». A service not in the directory is not claimed. The directory is reviewed at least annually (the organisation's adopted cycle) and whenever a service is added, suspended or withdrawn.

5.2 Diagnostic and treatment capability with qualified personnel

Each defined clinical service (broad speciality and super speciality) has:

- diagnostic and treatment services appropriate to that clinical service;
- suitably qualified medical, nursing and paramedical staff for the patient's clinical needs;
- out-patient services, in-patient services, day-care where the service uses day-care, and emergency cover by the consultant(s).

Infrastructure for diagnostics and treatment follows regulatory requirements and professional-body guidance where available. The Medical Superintendent ensures personnel qualifications are verified at appointment and kept current. Where a service limitation exists, it is written in the department scope (section 5.3) and reflected in the display (section 5.4).

5.3 Define scope of clinical services of each department

Each department — super speciality, broad speciality and diagnostic — has a written scope. Scope may be by inclusion or by exclusion relative to services practised in the department. Example content for a nephrology department could include biopsy, shunts, fistulas, haemodialysis and CAPD where those are practised.

All clinical and diagnostic outsourced services are documented. Staff know what is outsourced. That information is also available to patients through the website, display boards and department brochures as the hospital chooses.

The Medical Superintendent approves each scope. The Quality Coordinator holds the current set. Scopes are reviewed at least annually (the organisation's adopted cycle) and on any material change in capability.

5.4 Prominent permanent bi-lingual display of defined clinical services

The display states the names of clinical and diagnostic departments of «Hospital Name». It is permanent (board, citizen's charter or equivalent; electronic display is allowed as a supplement or alternative where it remains permanently visible). It is placed where patients and visitors can see it — at least at the main entrance and the registration area.

The display is at least bi-lingual: the State language or the language spoken by the majority of people in the catchment, and English. Brochures, standees and the website may supplement the permanent display; they do not replace it.

When a service is added or withdrawn, the display is updated within seven working days.

6. Governance and responsibility

Role	Responsibility
Medical Superintendent	Accountable that healthcare services are defined, resourced and displayed. Approves the service directory and each department clinical scope.
Heads of departments	Define and maintain the clinical scope of their department, including outsourced elements. Ensure diagnostic and treatment capability and qualified personnel for out-patient, in-patient, day-care and emergency cover as applicable.
Quality Coordinator	Holds the service directory and department scope statements.
Registration / front-office staff	Direct patients only to services listed in the current directory and display.
All clinical staff	Report any mismatch between displayed and actual services the same shift.

7. Quality monitoring (RCA → CAPA)

What is monitored:

- Service directory is current and matches what is actually provided.
- Each listed clinical service has diagnostic and treatment capability and qualified personnel recorded, including day-care and emergency cover where applicable.
- Department scope statements exist, are approved, and document outsourced clinical and diagnostic services.
- Permanent bi-lingual display is legible, current and matches the service directory at every display point.

Root-cause analysis is required when the same service-display mismatch recurs; the organisation's defined recurrence threshold applies, as documented in the hospital's Root Cause Analysis procedure.

This policy is reviewed annually, and sooner when services are added, suspended or withdrawn.

8. Training and staff acknowledgement

All staff are informed of this policy at induction and once a year after that. Training covers the service directory, department scopes, outsourced services, how to check the display, and how to report a mismatch.

Staff acknowledgement

I have read the Policy on Defined and Displayed Healthcare Services of «Hospital Name». I will follow the processes described.

Name: _____ Designation: _____

Department / floor: _____ Date: _____

Signature: _____

(One row per staff member. The Quality Coordinator holds signed acknowledgements with the induction record.)

9. Distribution

Official master copy: office of the Medical Superintendent, «Hospital Name», with the Quality Coordinator.

Copies issued to: registration; every clinical and diagnostic department; out-patient; emergency; day-care; nursing administration.

The current version is available to all staff at the front-office policy file and, if the hospital keeps an intranet, at staff intranet / policies.

When a new version is issued, take old copies out of use.

10. Abbreviations

Abbreviation	Meaning
AAC	Access, Assessment and Continuity of Care (NABH Hospitals chapter)
CAPA	corrective and preventive action
CAPD	continuous ambulatory peritoneal dialysis
HCO	Hospital (Full Accreditation programme under NABH Hospitals 6th Edition)
NABH	National Accreditation Board for Hospitals and Healthcare Providers
OE	objective element
OPD	out-patient department

11. Traceability to NABH HCO Full Accreditation 6th Edition AAC.1

This table is an index. It is not how the policy is organised. An asterisk in the Level column means documentation of the process is required.

OE	Level	Requirement	Where addressed	Responsible
AAC.1.a	Commitment	The healthcare services being provided are defined and are in consonance with the needs of the community.	Section 3; 5.1	Medical Superintendent (approve); Heads of departments (define); Quality Coordinator (hold directory)
AAC.1.b	Commitment	Each defined clinical service shall have diagnostic and treatment services with suitably qualified personnel who provide out-patient, in-patient, daycare and emergency cover.	Section 3; 5.2	Heads of departments (ensure capability); Medical Superintendent (verify qualifications)
AAC.1.c	Commitment*	Scope of the clinical services of each department is defined.	Section 3; 5.3	Heads of departments (write scope); Medical Superintendent (approve); Quality Coordinator (hold)
AAC.1.d	Commitment	The organisation's defined clinical services are prominently displayed.	Section 3; 5.4	Quality Coordinator (maintain display); registration staff (direct patients per directory)

12. Required Records / Evidence Checklist

Records the hospital holds under this policy, listed by objective element.

AAC.1.a — Healthcare services defined in consonance with community needs

- Service directory listing every healthcare service provided.
- Evidence that community needs informed service planning (feedback, referral, disease-burden or similar).
- Minutes or record of annual review of the service directory.

AAC.1.b — Diagnostic and treatment capability with qualified personnel

- Service directory entry for each clinical service showing diagnostic capability, treatment capability and personnel.
- Personnel qualification and verification records for medical, nursing and paramedical staff per service.
- Record of out-patient, in-patient, day-care and emergency cover arrangements for each defined clinical service.
- Record of any service limitation and the defined referral pathway.

AAC.1.c — Scope of clinical services of each department defined

- Written clinical scope statement for each department (super speciality, broad speciality and diagnostic), approved and dated.
- Documented list of clinical and diagnostic outsourced services known to staff.
- Record of annual or change-triggered scope review.
- Brochure or equivalent detailing department scope where the hospital uses one.

AAC.1.d — Clinical services prominently displayed

- Photographs or records of the permanent display at each mandated location.
- Evidence the display is at least bi-lingual (State/majority language and English).
- Record of display update when services changed, with date of update.

13. References

- National Accreditation Board for Hospitals and Healthcare Providers (NABH), Guidebook to Accreditation Standards for Hospitals, 6th Edition — Access, Assessment and Continuity of Care (AAC), standard AAC.1.

Disclaimer

This document is a template prepared for the guidance of «Hospital Name» and must be reviewed, adapted and formally approved by «Hospital Name» before use. Every entry marked _____ must be completed before the document is issued.

The requirements in this document are accreditation requirements of the NABH Accreditation Standards for Hospitals, 6th Edition, not duties under a named Act of Parliament. This policy does not import the Consumer Protection Act, 2019, the Clinical Establishments Act, 2010, or the Mental Healthcare Act, 2017 as a checklist. Statutory duties that arise under other documents of «Hospital Name» remain those documents. «Hospital Name» is responsible for verifying any statutory duty that applies to it; this document does not constitute legal advice.

«Hospital Name» remains responsible for verifying that it is current and consistent with the edition of the accreditation standard against which it is being assessed. The clinical and technical content reflects recognised national and international guidance current at the date of preparation.

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